

PRIOR AUTHORIZATION REQUEST

Cover Letter

Patient Name	John Smith
Date of Birth	1968-03-15
Insurance	United Healthcare
Member ID	UHC-887612
Policy Number	UHC-POL-2026-001
CPT Code	75563
ICD-10	I42.0
Physician	Dr. Ramesh Kumar
Physician NPI	1234567890
Facility	City Heart Institute
Date of Request	March 31, 2026

Dear Prior Authorization Review Team,

This letter serves as a formal request for prior authorization for a Cardiac MRI with Contrast (CPT Code: 75563) for our patient, John Smith (DOB: 1968-03-15, Male). Mr. Smith is insured under United Healthcare, Member ID: UHC-887612, Policy Number: UHC-POL-2026-001, Group Number: GRP-7721, Plan Name: UHC Gold Plus.

The primary diagnosis for this request is Dilated Cardiomyopathy (ICD-10: I42.0). Mr. Smith presents with severely reduced Left Ventricular Ejection Fraction (LVEF) of 35% as documented in a recent echocardiogram (January 15, 2026) and persistently elevated BNP levels (450 pg/mL). Despite being on optimal guideline-directed medical therapy for over three years, he continues to experience NYHA Class III symptoms and reduced functional capacity.

The Cardiac MRI is medically necessary for further tissue characterization of the myocardium, assessment of myocardial fibrosis, differentiation of ischemic versus non-ischemic cardiomyopathy, and to guide critical treatment decisions, including potential ICD implantation. The echocardiogram findings, while indicative of severe dysfunction, require the advanced detail provided by MRI for comprehensive management.

All supporting clinical documentation, including recent echocardiogram report, laboratory results, physician order, and detailed prior treatment records, has been submitted for your review. The requesting physician is Dr. Ramesh Kumar, NPI: 1234567890, a Board-Certified Cardiologist at City Heart Institute.

We kindly request your prompt review and approval for this essential procedure.

Sincerely,

Dr. Ramesh Kumar, MD

Cardiology

City Heart Institute

NPI: 1234567890

Date: March 31, 2026

Clinical Summary

DIAGNOSIS

Patient John Smith, a 58-year-old male, has a confirmed diagnosis of Dilated Cardiomyopathy (ICD-10: I42.0) and Heart Failure with Reduced Ejection Fraction (ICD-10: I50.20).

PATIENT HISTORY

Mr. Smith has a history of Dilated Cardiomyopathy for at least three years, during which he has been managed with optimal guideline-directed medical therapy (GDMT). Current medications include Carvedilol 25mg BID, Lisinopril 10mg QD, Spironolactone 25mg QD, Furosemide 40mg QD, Aspirin 81mg QD, and Atorvastatin 40mg QD. Previous attempts with Sacubitril/Valsartan and Ivabradine were discontinued due to symptomatic hypotension/dizziness and inadequate heart rate reduction, respectively. Despite this extensive therapy, Mr. Smith continues to exhibit persistent LVEF of 35%, NYHA Class III symptoms, and reduced functional capacity.

REASON FOR PROCEDURE

A Cardiac MRI with Contrast (CPT 75563) is requested to provide advanced myocardial tissue characterization. This is crucial for assessing myocardial fibrosis (late gadolinium enhancement), differentiating between ischemic and non-ischemic etiologies of cardiomyopathy, evaluating myocardial viability, and guiding future treatment decisions, such as the potential need for an implantable cardioverter-defibrillator (ICD).

SUPPORTING EVIDENCE

1. ****Echocardiogram Report (January 15, 2026):**** Documents severely reduced LVEF of 35%, moderately dilated left ventricle (LV End Diastolic Diameter: 6.2 cm, LV End Systolic Diameter: 5.1 cm), diffuse global hypokinesis, Grade II diastolic dysfunction, mild functional mitral regurgitation, and a small pericardial effusion. The report recommends Cardiac MRI for further tissue characterization.
2. ****Laboratory Results Report (approx. March 10, 2026):**** Shows significantly elevated BNP at 450 pg/mL and NT-proBNP at 1850 pg/mL, consistent with moderate to severe heart failure. Renal function (Creatinine 1.1 mg/dL, eGFR 72 mL/min) and CBC are within normal limits.
3. ****Prior Treatment Records (March 16, 2026):**** Confirms over three years of GDMT with persistent LVEF of 35%, NYHA Class III symptoms, and elevated BNP, indicating inadequate clinical response.

CLINICAL JUSTIFICATION

Given Mr. Smith's persistent severe LV systolic dysfunction and ongoing heart failure symptoms despite optimal and prolonged GDMT, further characterization of his myocardium is essential. While the echocardiogram provides valuable information, a Cardiac MRI is necessary to precisely assess myocardial fibrosis, viability, and differentiate the underlying etiology of his dilated cardiomyopathy. This detailed information is critical for refining his prognosis and guiding advanced therapeutic interventions, ensuring the most appropriate and effective management strategy.

Requirements Checklist

Requirement	Status	Evidence
Detailed note from treating Cardiologist explaining medical necessity. Must include: patient history, current symptoms, previous treatments tried, reason echocardiogram is insufficient. Must be signed and dated.	✗ MISSING	The uploaded 'Clinical Justification Note' document (Clinical_Justification_Note_Kumar.pdf) is a header/footer only and lacks specific patient data, history, symptoms, previous treatments, or reason echocardiogram is insufficient as required by policy. While the information is present across other documents, the specific 'Clinical Justification Note' file is incomplete.
Most recent echocardiogram report within last 12 months. Must document LVEF value, chamber dimensions, wall motion analysis, and valve function.	✓ MET	Transthoracic Echocardiogram Report, January 15, 2026: LVEF 35%, LV End Diastolic Diameter 6.2 cm, LV End Systolic Diameter 5.1 cm, Diffuse global hypokinesis, Mild functional mitral regurgitation.
Laboratory results within last 30 days including: BNP or NT-proBNP levels, complete metabolic panel, complete blood count.	✓ MET	Laboratory Results Report (approx. March 10, 2026): BNP 450 pg/mL, NT-proBNP 1850 pg/mL, Complete Metabolic Panel: Normal, Complete Blood Count: Normal.
Official order on physician letterhead signed by referring Cardiologist. Must include: procedure requested, ICD-10 diagnosis code, clinical indication, physician NPI number.	✓ MET	Physician Order Prior Auth Cardiac MRI, March 16, 2026: Cardiac MRI with Contrast (CPT 75563) requested for Dilated Cardiomyopathy (I42.0). Signed by Dr. Ramesh Kumar, NPI 1234567890.
Documentation of cardiac medications tried for minimum 3 months including drug names, dosages, duration of therapy, and patient response or reason for discontinuation.	✓ MET	Cardiac Medication History Report, March 16, 2026: Documents 3 years of GDMT including Carvedilol, Lisinopril, Spironolactone, Furosemide, with details on dosages, duration, and response/discontinuation reasons for Sacubitril/Valsartan and Ivabradine.
Patient must have a confirmed diagnosis of one of the following conditions: Dilated Cardiomyopathy (ICD-10: I42.0), Hypertrophic Cardiomyopathy (ICD-10: I42.1), Restrictive Cardiomyopathy (ICD-10: I42.5), Cardiac Sarcoidosis (ICD-10: D86.85), or Suspected Cardiac Tumor or Mass.	✓ MET	Primary diagnosis: Dilated Cardiomyopathy (ICD-10: I42.0) confirmed by Physician Order and Echocardiogram Report.
Symptoms must be present and documented for a minimum period of 3 months including: Shortness of breath on exertion, Chest pain or pressure, Palpitations or arrhythmia, Unexplained syncope or near-syncope, or Reduced exercise tolerance.	✓ MET	Prior Treatment Records, March 16, 2026, and Physician Order, March 16, 2026: Patient has NYHA Class III symptoms and reduced functional capacity for at least 3 years, consistent with shortness of breath on exertion.

Patient must have completed echocardiogram prior to MRI request. Echocardiogram must show: LVEF (Left Ventricular Ejection Fraction) documented value, AND Evidence of wall motion abnormality OR Structural cardiac abnormality requiring further characterization.	✓ MET	Transthoracic Echocardiogram Report, January 15, 2026: LVEF 35% documented. Evidence of diffuse global hypokinesis (wall motion abnormality) and moderately dilated LV with mild functional mitral regurgitation (structural abnormality).
Request must be ordered by a board-certified Cardiologist or Cardiac Electrophysiologist.	✓ MET	Requesting Physician: Dr. Ramesh Kumar MD, Board Certified Cardiologist (Physician Order Prior Auth Cardiac MRI).

Attached Documents

The following documents are attached to this package:

#		
1	Clinical Justification Note Kumar	Clinical_Justification_Note_Kumar.pdf
2	Echocardiogram Report Jan2026	Echocardiogram_Report_Jan2026.pdf
3	Laboratory Results Report	Laboratory_Results_Report.pdf
4	Physician Order Prior Auth Cardiac Mri	Physician_Order_Prior_Auth_Cardiac_MRI.pdf
5	Prior Treatment Records Cardiac Medication	Prior_Treatment_Records_Cardiac_Medication.pdf

CITY HEART INSTITUTE

Department of Cardiology | Patient Clinical Notes

Patient Name: John Smith Member ID: UHC-887612 Physician: Dr. Ramesh Kumar
DOB: 03/15/1968 Gender: Male Policy #: UHC-POL-2026-001 NPI: 1234567890
Insurance: United Healthcare Date: 03/01/2026 Specialty: Cardiology

Chief Complaint / Presenting Hx:

57 yr old ♂ w/ progressive dyspnea on exertion x3 months, bilateral ankle edema + orthopnea. No chest pain. PMHx: HTN, DM type 2. Meds: metoprolol, lisinopril, metformin.

Diagnosis:

Dilated Cardiomyopathy ICD-10: I42.0

Examination Findings:

BP: 148/92 mmHg HR: 88 bpm RR: 18/min SpO₂: 96%
Elevated JVP ++. S3 gallop heard at apex. Bilateral basal crepitations. Pitting pedal edema 2+. No murmur.

Investigations / Labs:

BNP: 450 pg/mL ↑ (nl <100)
Metabolic Panel: WNL ✓
ECG: Sinus rhythm, LVH pattern, non-specific ST changes.
Echo (prelim): EF ~30-35%. Diffuse LV hypokinesis.

Procedure Ordered:

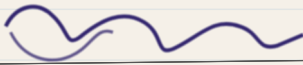
Cardiac MRI with Contrast CPT: 75563
Facility: City Heart Institute — Cardiology Dept.
Indications: assess myocardial viability, EF quantification, exclude ischemic etiology, fibrosis mapping (LGE).

Plan / Management:

- 1) Admit for monitoring + IV diuresis (Lasix 40mg BD)
- 2) Optimise GDMT — add spironolactone, uptitrate BB
- 3) Cardiac MRI as above (pre-auth UHC Gold Plus req'd)
- 4) Cardiology f/u in 2 wks post-MRI
- 5) Pt counselled re: sodium restriction, fluid balance, weight monitoring. ICD discussion deferred.

Allergies: NKDA

Group: GRP-7721 Plan: UHC Gold Plus

Physician Signature: 

Date signed: 01/03/2026 Dr. Ramesh Kumar, MD | Cardiology | NPI: 1234567890



TRANSTHORACIC ECHOCARDIOGRAM REPORT

Ordering Physician:

Dr. Ramesh Kumar MD

Performing Physician:

Dr. Sarah Johnson MD

Facility:

City Heart Institute

Clinical Indication:

Dilated Cardiomyopathy; Follow up evaluation

Study Date:

January 15, 2026

Report Date:

January 15, 2026

Study Type:

Complete TTE

SECTION 1: TECHNICAL QUALITY

Study Quality: Adequate

Windows: Acceptable acoustic windows obtained.

Views: Parasternal long axis, short axis, apical 4-chamber, 2-chamber, 3-chamber, and subcostal views were successfully obtained and analyzed.

SECTION 2: LEFT VENTRICLE MEASUREMENTS

Measurement	Result	Normal Reference Range
LV End Diastolic Diameter	6.2 cm	Less than 5.5
LV End Systolic Diameter	5.1 cm	Less than 3.9
Interventricular Septum	0.8 cm	0.6 to 1.1
Posterior Wall Thickness	0.8 cm	0.6 to 1.1
LV Ejection Fraction (LVEF)	35%	Greater than 55%
LV Fractional Shortening	18%	25 to 45%

SECTION 3: LEFT VENTRICLE FUNCTION

LVEF: 35% – Severely reduced.

Wall Motion: Diffuse global hypokinesis noted. No regional wall motion abnormalities (RWMA) identified.

Diastolic Function: Grade II diastolic dysfunction (pseudonormal pattern).

LV Size: Moderately dilated left ventricle.

SECTION 4: RIGHT VENTRICLE

RV Size: Normal.

RV Function: Mildly reduced.

TAPSE: 1.6 cm (mildly reduced).

RV Systolic Pressure: Estimated 38 mmHg.

SECTION 5: VALVES

Mitral Valve: Mild mitral regurgitation identified. Etiology appears functional, secondary to LV dilation and annular stretching.

Aortic Valve: Trileaflet, normal opening. No significant stenosis or regurgitation visualized.

Tricuspid Valve: Mild tricuspid regurgitation.

Pulmonic Valve: Normal appearance and function.

SECTION 6: OTHER FINDINGS

Pericardium: Small pericardial effusion noted.

Aorta: Normal aortic root diameter.

IVC: Dilated IVC suggesting elevated right atrial pressure.

SECTION 7: CONCLUSION

1. Severely reduced left ventricular systolic function with LVEF of 35%.
2. Moderately dilated left ventricle consistent with dilated cardiomyopathy.
3. Diffuse global hypokinesis.
4. Mild mitral regurgitation — functional etiology.
5. Grade II diastolic dysfunction.
6. Small pericardial effusion.
7. Recommend Cardiac MRI for further tissue characterization.

Interpreting Physician: Dr. Sarah Johnson MD

Board Certified Echocardiographer

Date: January 15, 2026

Ordering Physician:	Dr. Ramesh Kumar MD	Specimen Type:	Venous Blood
Collection Date:	March 10, 2026	Clinical Indication:	Dilated Cardiomyopathy
Report Date:	March 10, 2026		Cardiac MRI Pre-authorization

SECTION 1 - CARDIAC BIOMARKERS

Test	Result	Unit	Reference Range	Flag
BNP (B-type Natriuretic Peptide)	450	pg/mL	Less than 100	HIGH
NT-proBNP	1850	pg/mL	Less than 125 (age < 75)	HIGH
Troponin I (High Sensitivity)	0.04	ng/mL	Less than 0.04	BORDERLINE

SECTION 2 - COMPLETE BLOOD COUNT

Test	Result	Unit	Reference Range	Flag
WBC	7.2	10^3/uL	4.5 to 11.0	NORMAL
RBC	4.8	10^6/uL	4.5 to 5.9	NORMAL
Hemoglobin	13.8	g/dL	13.5 to 17.5	NORMAL
Hematocrit	41.2	%	41 to 53	NORMAL
Platelets	215	10^3/uL	150 to 400	NORMAL
MCV	88	fL	80 to 100	NORMAL

SECTION 3 - COMPREHENSIVE METABOLIC PANEL

Test	Result	Unit	Reference Range	Flag
Sodium	138	mEq/L	136 to 145	NORMAL
Potassium	4.1	mEq/L	3.5 to 5.1	NORMAL
Creatinine	1.1	mg/dL	0.7 to 1.3	NORMAL
eGFR	72	mL/min	Greater than 60	NORMAL
BUN	18	mg/dL	7 to 20	NORMAL

Test	Result	Unit	Reference Range	Flag
Glucose	95	mg/dL	70 to 100	NORMAL
AST	28	U/L	10 to 40	NORMAL
ALT	22	U/L	7 to 56	NORMAL
Albumin	3.8	g/dL	3.5 to 5.0	NORMAL

SECTION 4 - LIPID PANEL

Test	Result	Unit	Reference Range	Flag
Total Cholesterol	195	mg/dL	Less than 200	NORMAL
LDL	118	mg/dL	Less than 130	NORMAL
HDL	42	mg/dL	Greater than 40	NORMAL
Triglycerides	175	mg/dL	Less than 150	BORDERLINE HIGH

SECTION 5 - INTERPRETATION NOTE

Significantly elevated BNP at 450 pg/mL and NT-proBNP at 1850 pg/mL consistent with moderate to severe heart failure.
Renal function preserved. Electrolytes within normal limits.
Results support ongoing heart failure management and prior authorization for advanced cardiac imaging.

Authorized By:

Dr. Michael Chen MD
Board Certified Pathologist
Date: March 10, 2026

CITY HEART INSTITUTE

Department of Cardiology | 123 Medical Center Drive | Houston, Texas 77001

Phone: 555-123-4567 | Fax: 555-123-4568 | www.cityheartinstitute.com

Date: March 16, 2026

To:

United Healthcare Prior Authorization Department

Radiology Prior Authorization Team

From:

Dr. Ramesh Kumar MD

Board Certified Cardiologist

NPI: 1234567890 | License: MD-12345-TX | DEA: RK1234563

SUBJECT: PRIOR AUTHORIZATION REQUEST

PROCEDURE: CARDIAC MRI WITH CONTRAST (CPT 75563)

ICD-10 CODE: I42.0

REQUESTED PROCEDURE

Name:	Cardiac MRI with Contrast
CPT Code:	75563
Urgency:	Routine — Non Emergency
Facility:	City Heart Institute Radiology
Estimated Date:	Within 30 days of approval

CLINICAL INDICATION

Primary Diagnosis: Dilated Cardiomyopathy (ICD-10: I42.0)

Secondary Diagnosis: Heart Failure with Reduced Ejection Fraction (ICD-10: I50.20)

MEDICAL NECESSITY STATEMENT

This procedure is medically necessary for:

- Tissue characterization of myocardium
- Assessment of myocardial fibrosis via late gadolinium enhancement imaging
- Differentiation of ischemic versus non-ischemic cardiomyopathy
- Assessment of myocardial viability
- Guidance of treatment decisions including consideration of ICD implantation

SUPPORTING CLINICAL INFORMATION

- LVEF 35% on recent echocardiogram
- Elevated BNP 450 pg/mL
- NYHA Class III symptoms
- Optimal medical therapy ongoing
- No contraindications to MRI or gadolinium contrast media

I certify that this procedure is medically necessary and appropriate for this patient. All information provided is accurate and complete to the best of my knowledge.

Dr. Ramesh Kumar MD
Board Certified Cardiologist
Date: March 16, 2026
NPI: 1234567890
Contact: 555-123-4567

Treating Physician:	Dr. Ramesh Kumar MD	Report Date:	March 16, 2026
Facility:	City Heart Institute	Specialty:	Cardiology
Period Covered:	Jan 2023 – Mar 2026	Clinical Indication:	Dilated Cardiomyopathy

This document summarizes the complete cardiac medication history for a patient with confirmed Dilated Cardiomyopathy (ICD-10: I42.0) over a period of 3 years from January 2023 to March 2026. All medications listed below were prescribed and monitored by the treating cardiologist as part of optimal medical therapy for heart failure with reduced ejection fraction.

SECTION 1 - MEDICATION HISTORY TABLE

Medication	Dose/ Freq	Dates	Duration	Response	Status
Carvedilol (Beta Blocker)	25mg Twice daily	Jan 2023 – Present	3 years	Partial response – HR controlled but symptoms persist	CURRENT
Lisinopril (ACE Inhibitor)	10mg Once daily	Jan 2023 – Present	3 years	Partial response – BP controlled but LVEF unchanged	CURRENT
Spirolactone (Aldo. Antagonist)	25mg Once daily	Mar 2023 – Present	2y 9mo	Minimal improvement in symptoms	CURRENT
Furosemide (Loop Diuretic)	40mg Once daily	Mar 2023 – Present	2y 9mo	Adequate fluid control; edema managed	CURRENT
Sacubitril/ Valsartan (ARNI)	24/26mg Twice daily	Sep 2023 – Nov 2023	2 months	Discontinued due to symptomatic hypotension and dizziness	DISCONTINUED
Ivabradine	5mg Twice daily	Jan 2024 – Mar 2024	2 months	Discontinued – inadequate heart rate reduction	DISCONTINUED

SECTION 2 - TREATMENT RESPONSE SUMMARY

Despite 3 years of optimal medical therapy with maximum tolerated doses of guideline directed medical therapy (GDMT) including beta blocker, ACE inhibitor, and aldosterone antagonist, patient continues to exhibit:

- Persistent LVEF of 35%
- NYHA Class III symptoms (marked limitation in activity)
- Elevated BNP levels (refer to lab report dated 03/10/2026)
- Reduced functional capacity

SECTION 3 - CURRENT MEDICATION LIST

- **Carvedilol:** 25mg twice daily
- **Lisinopril:** 10mg once daily
- **Spironolactone:** 25mg once daily
- **Furosemide:** 40mg once daily
- **Aspirin:** 81mg once daily
- **Atorvastatin:** 40mg once daily

Physician Attestation:

I certify that the above medication history is accurate and complete. The patient has received optimal guideline directed medical therapy for a minimum of 3 years with inadequate clinical response. Advanced cardiac imaging (Cardiac MRI) is required to guide further treatment decisions, including viability assessment and potential ICD placement.

Dr. Ramesh Kumar MD

Board Certified Cardiologist | NPI: 1234567890

Date: March 16, 2026